



INTERVIEW PACKET

AbbVie Interview Prep

Director, Government Affairs (U.S. Reimbursement)

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Built from resume evidence, role description, and AbbVie talking-point notes on March 30, 2026.

Primary resume source: Joseph_Stewart_Resume copy 2.pdf

Federal and state policy

Advocacy depth across federal and state reimbursement issues.

Manufacturer strategy

Direct internal strategy experience from Otsuka.

Biosimilars and payer dynamics

AAM and consulting work sharpen the channel and access lens.

Policy to access

Core differentiator: policy -> ASP -> provider economics -> access.

Role Priorities

The AbbVie posting is looking for someone who can own Medicare reimbursement strategy at the product level, represent the company before CMS, and tie policy developments to launch and access decisions.

Role mandate from the AbbVie posting	What you should signal in the interview
- Monitor and track reimbursement policy developments - Develop comprehensive product coverage and reimbursement strategies for Medicare - Identify Medicare Part B policy threats and opportunities that affect AbbVie's portfolio - Represent AbbVie's interests before CMS and other relevant federal agencies - Partner across Government Affairs to build advocacy strategies aligned with enterprise goals and product-specific reimbursement strategies - Grow deep understanding of on-market products, pipeline assets, business strategies, and channel-specific goals	- Product-specific Medicare strategy - CMS engagement depth - ASP and physician-administered drug fluency - Cross-functional leadership - Oncology and neuroscience awareness - Ability to turn policy into access execution
Core qualifications emphasized	Best proof points from your background
- Deep knowledge of the U.S. federal reimbursement environment and familiarity with the commercial payer market - 10 or more years of relevant experience; Government Affairs experience preferred - Expertise in coding, coverage, reimbursement, and Average Sales Price methodology - Previous experience launching physician-administered drugs - Experience in oncology and neuroscience reimbursement issues preferred - Knowledge of Medicare and Medicaid with emphasis on Medicare Parts A and B, the discarded drug refund policy, and the Inflation Reduction Act	- Consulting work on reimbursement risk, ASP, and pricing strategy - Otsuka leadership in reimbursement and health policy - AAM work on biosimilars commercialization and provider incentives - Federal lobbying and agency-facing policy work - Portfolio perspective across manufacturers, PBMs, payers, biosimilars, and generics

Resume-to-Role Match

Role need	Resume evidence	How to frame it in conversation
Medicare Part B, ASP, and provider economics	Current consulting work includes reimbursement models using ASP, provider acquisition cost, and rebate structures. AAM work focused on provider disincentives under ASP-based reimbursement.	Lead with the idea that coverage alone does not drive uptake; provider economics does.
Manufacturer plus advocacy perspective	Otsuka reimbursement and health policy leadership combines manufacturer strategy with field government affairs execution. Earlier federal lobbying work adds direct policy engagement experience.	Show you can translate external policy into internal action and represent the company credibly before CMS.

Role need	Resume evidence	How to frame it in conversation
Oncology and neuroscience relevance	AAM work included oncology portfolios and biosimilar commercialization issues. Otsuka experience strengthens the neuroscience and access narrative.	Tie your background to AbbVie's oncology growth and neuroscience complexity without overstating therapeutic ownership.
Cross-functional strategy execution	Consulting work bridges policy, commercial, and market access teams. Otsuka role included oversight of an eight-person field-based government affairs team.	Emphasize product-specific strategy, stakeholder alignment, and execution under deadlines.

This packet is built for interview use. The goal is to present a clear narrative that connects reimbursement policy to product-level strategy, provider behavior, and access outcomes.

Positioning Summary

I operate at the intersection of reimbursement policy, pricing, and real-world access. My focus is not just understanding policy, but translating it into product-level strategy, provider behavior, and adoption outcomes.

- Federal and state policy plus advocacy experience
- Manufacturer strategy experience from Otsuka
- Biosimilars and payer dynamics experience from AAM
- Consulting work across manufacturers, PBMs, payers, biosimilars, and generics

Differentiator: Policy -> ASP -> provider economics -> access

AbbVie Portfolio Context

AbbVie has major exposure across immunology, oncology, and neuroscience. Historically the portfolio has had meaningful Part D relevance, but the strategic risk and opportunity set is shifting toward Part B oncology and pipeline assets where provider economics become more central.

- Immunology anchors: Humira, Skyrizi, Rinvoq
- Oncology anchors: Imbruvica, Venclexta
- Neuroscience anchor: Vraylar

Medicare Part B: Core to the Role

For this role, the real issue is not only whether a product is covered. The real issue is whether reimbursement mechanics support consistent provider adoption.

- ASP plus 6 percent, with the effective add-on closer to roughly 4.3 percent after sequestration
- Buy-and-bill economics drive adoption behavior

- 340B can distort spread and create uneven incentives
- Site-of-care management can redirect utilization even when coverage remains intact

"Coverage is not the barrier, provider economics is."

Adoption depends on acquisition cost versus ASP reimbursement, margin stability, and operational burden. Even favorable policy fails commercially when physician office or hospital economics do not work.

IRA Impact

The Inflation Reduction Act changes the center of gravity from pure price optimization to access and lifecycle optimization.

- Maximum Fair Price negotiation matters immediately for AbbVie through Imbruvica
- Inflation rebates constrain price growth
- Part D redesign changes plan, manufacturer, and beneficiary incentives

The critical implication for AbbVie is that IRA pressure does not stop at a list-price discussion. It can cascade into reimbursement mechanics and provider behavior.

Part B Plus IRA Intersection

ASP is directly tied to price. If IRA-driven pricing lowers price, it can lower ASP. In buy-and-bill settings that can compress provider margin even when coverage is unchanged.

"If IRA-driven pricing reduces ASP, it can compress provider margins and create access friction in buy-and-bill settings, even when coverage is unchanged."

This is a high-value point because it shows how a federal pricing policy can translate into provider-level access friction without any formal coverage restriction.

CMS Strategy

CMS engagement should be sequenced, product-specific, and tied to real reimbursement milestones.

- Coding: HCPCS and any related coding pathway work
- Payment: ASP mechanics, payment benchmark strategy, and launch timing
- Coverage: NCD, LCD, MAC behavior, and claims administration risk
- Advocacy: comments, education, and stakeholder engagement timed to rulemaking cycles

"CMS engagement must be sequenced and product-specific, not siloed."

Provider Economics: Interview Edge



Most candidates will speak at the policy level. The stronger answer is to show how policy becomes provider behavior.

- ASP lag versus WAC at launch
- 340B incentives and spread distortion
- Site-of-care migration pressure
- Hospital versus physician office margin differences
- Operational burden in billing, acquisition, and utilization management

"Even when policy is favorable, adoption fails if provider economics do not work."

Policy Framework

- IRA negotiation reduces pricing flexibility over time
- Inflation rebates limit price growth
- PBM reform changes where value extraction occurs across channels

The manufacturer response is lifecycle strategy plus access optimization. The payer and PBM response is to shift value capture across benefit design, channel strategy, and utilization management.

Likely Questions

How do you approach Medicare policy?

I translate policy into reimbursement mechanics, model product-level impact, align internal stakeholders, and then execute an advocacy strategy that is tied to product-specific business goals rather than general policy preferences.

What is the IRA impact on AbbVie?

The immediate exposure is Imbruvica. The medium-term issue is broader oncology and Part B exposure. The structural issue is that IRA pressure can flow through ASP and compress provider margin, which creates access friction in buy-and-bill settings.

Why are you a strong fit?

I focus on how policy actually plays out in pricing, provider behavior, and access. That allows me to not just interpret policy, but build strategy around it.

Why Me Scripts

30-second script

I bring a reimbursement lens that starts with policy but does not stop there. My background spans federal and state advocacy, manufacturer strategy, biosimilars policy, and consulting across manufacturers and payers. What makes me different is that I connect CMS policy to ASP, provider economics, and access outcomes, which is exactly where AbbVie needs strategic depth as Part B exposure grows.

90-second script

What makes me a strong fit for this role is that I have worked across the full chain from policy development to commercial consequence. In consulting, I advise clients on reimbursement risk, ASP dynamics, provider acquisition cost, and market access strategy. At Otsuka, I translated reimbursement and policy developments into actionable recommendations for internal stakeholders and led an eight-person field-based government affairs team. At the Association for Accessible Medicines, I worked directly on biosimilars commercialization and the reimbursement barriers that shape real uptake, including provider disincentives under ASP-based reimbursement. Earlier in my career, I worked as a federal lobbyist, which gave me direct experience with legislative language, agency engagement, and coalition strategy. Taken together, that means I can represent AbbVie credibly before CMS while also building product-specific reimbursement strategies that make sense for brand teams, market access teams, and providers.

Why AbbVie and why this role?

AbbVie is at a point where reimbursement strategy has to be deeply integrated with enterprise portfolio decisions. The company has meaningful exposure in oncology and neuroscience, and the policy environment is changing quickly under IRA, Part B pressure, and broader federal scrutiny. This role sits exactly at the point where external CMS policy needs to be translated into product strategy, launch planning, and access execution. That is the work I have been doing across consulting, manufacturer, and trade association settings, so the role feels like a direct match for my experience.

Strong closing line

AbbVie needs someone who can connect CMS policy to real reimbursement strategy and product access. That is the core of my experience.